

Trauma

T-09 Burns BLS

Revised 3/1/2000

PRIORITIES:

- ABCs
- Assume airway/respiratory involvement in chemical burns and fires in close spaces
- Stop the burning process
- Search for associated injuries
- Assure an advanced life support response

Burns

Damage to the skin caused by contact with caustic material (chemical burns), electricity or fire. Second or third degree burns involving 20% of the body surface area or those associated with respiratory involvement are considered major burns.

1. Keeping safety in mind, remove patient to a safe area;
2. Ensure a patent airway;
3. Stop the burning process. Remove contact with agent unless adherent (e.g., hot tar). Brush off chemical powders then flush copiously with cool water. Apply cool soaks to the wounds. Caution: large surface areas covered with cool soaks may lead to hypothermia. In those situations, keep soaks on only long enough to stop the burning process;
4. OXYGEN – 10 liters/minute or higher, via mask. Be prepared to support ventilation with appropriate airway adjuncts;
5. Protect the burned area;
 - Do NOT break blisters;
 - Cover with clean dressings or sheets;
 - Remove restrictive clothing/jewelry if possible;
 - Do NOT remove adherent materials. Cool the material with water.
6. Consider:
 - Assess for associated injuries;
 - Field burn treatments are associated with hypothermia, monitor the patient for associated signs and symptoms;

Chemical Burns:

- Flush immediately with copious amounts of water (up to 20 minutes as needed);
- If chemical is dry, brush off. Remove clothes and then flush with water;
- Identify the chemical;
- Look for associated respiratory burns;

Electrical Burns:

- Turn off power source of electricity if still in contact with patient;
- Assess entrance and exit wounds, cover with sterile dressings.

Tar Burns:

- Initially cool with water, maintain body temperature after primary cooling measures;
- Do NOT attempt to remove tar.

Trauma T-09 Burns

Priorities

ABC's

Assume airway/respiratory involvement in chemical burns and fires in closed spaces
Move patient to a safe area
Stop the burning process
Search for associated injuries
Early Transport
EARLY CONTACT OF RECEIVING HOSPITAL
TRANSPORT TO LOCAL EMERGENCY FACILITY

Stabilize airway

Oxygen – Titrate to SpO₂ >95%
Be prepared to support ventilations with appropriate adjuncts

Cardiac Monitor

IV/IO Access

Preferably on an unburned extremity

NS fluid challenge
500mL for <20% TBSA burned
1000mL for >20% TBSA burned

Repeat vital signs and continue NS boluses until SBP is >100

If pt has hx of heart failure or other signs of volume overload start NS fluid challenge at 250mL for <20% TBSA burned and 500mL for >20% TBSA burned.

Treatment Guidelines

1. Stop the burning process. Use cool dressings only long enough to stop the burning process.
2. Remove contact with burning agent unless it is an adherent substance (example: tar). DO NOT remove adherent materials.
3. Remove restrictive clothing and jewelry.
4. Brush off chemical powders and flush copiously with cool water
5. Cover the burned area with clean dressings or sheets.
6. DO NOT break blisters.

For pain relief

Refer to M-04 Pain Management
Contact Base Physician for additional orders

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Transport

DISRUPTED COMMUNICATIONS

In the event of a "disrupted communications" situation where a base hospital physician CANNOT be contacted for orders, Solano County Paramedics MAY NOT utilize the portions of this protocol requiring base physician orders AND must transport to the closest receiving facility